

Surgery for anal cancer

Types of anal cancer surgery include local excision and abdominoperineal resection.

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Anal cancer surgery

Most people with [anal cancer](#) are treated with [chemoradiation](#). This may be the only treatment they need.

But you may be offered surgery if you have a very small tumour in the [anal margin](#). This will only be if the surgeon can remove it without affecting how you pass stools (poo)

Your doctors may also offer you surgery:

- after chemoradiation, if the chemoradiation does not remove all the cancer
- if the cancer comes back after chemoradiation
- if you cannot have [radiotherapy](#) – for example, because you have had radiotherapy to the pelvis before
- to relieve a [blockage in the bowel \(bowel obstruction\)](#) before you have chemoradiation.

If your doctor thinks you need surgery, they will explain the type of surgery you need and how it will help.

Types of surgery

Local excision

This surgery only removes the area of the anus containing the cancer cells. It is sometimes used to treat small, early-stage cancers in the anal margin.

This operation will not usually affect the anal sphincter. This means you will not have long-term problems with bowel control afterwards.

Your doctors may recommend you have radiotherapy or chemoradiation after surgery. This is usually if there are some cancer cells in the tissue close to where the cancer was removed.

Abdominal perineal resection (APR)

This surgery removes the anus, rectum and part of the colon. Doctors usually only advise having an APR if:

- chemoradiation has not removed all the cancer
- the cancer has returned after treatment.

This operation can be done as either keyhole (laparoscopic) surgery or open surgery. This depends on the size of the tumour.

As well as the wound or wounds on your tummy, you will have a wound on your bottom where the anus has been closed. The anus may be closed using muscle, fat and skin from another part of the body. This is called a flap.

You will have a permanent stoma (colostomy) after this operation. This can be upsetting and it may take time to adjust to. Your surgeon and a stoma nurse specialist will talk to you before the operation to help you prepare. You can ask them any questions. Your nurse will give you lots of support, and there are organisations that can help. We have more information about [having a stoma](#), including organisations that can help.

Abdominal perineal resection

Image: The diagram shows the large bowel in the abdomen. The colon is shown in orange. The lower end of the colon, the rectum and the anus, are shown in white. This is the area removed during an abdominal perineal resection.

Stoma and stoma bag (colostomy)

Image: The diagram shows a person's chest down to the thighs. It shows the position of the bowel within the abdomen (tummy). On the left-hand side of the tummy, the bowel comes to the surface of the tummy. This is the stoma. The stoma is covered with a stoma bag that hangs down with a sealed end.

Having a stoma before chemoradiation

Before chemoradiation, some people may need to have an operation to create a stoma.

You may have a stoma to relieve symptoms in the following situations:

- The cancer is causing you to have problems controlling your bowel. This is called incontinence.
- There is an abnormal opening between the bowel and skin. An abnormal opening between 2 areas of the body is called a [fistula](#).
- There is a fistula between the bowel and another organ, such as the bladder or vagina.
- There is a risk of a fistula forming.
- The cancer is causing a [blockage in the bowel \(bowel obstruction\)](#).

After you have finished chemoradiation, your doctors will assess you to decide whether you can have the stoma reversed. This will mean you can pass stools through the anus again. It is not always possible to reverse the stoma.

Before your operation

Your operation will be carefully planned. For an APR, there may be different specialists involved. This might include a plastic surgeon and a gynaecologist.

You will go to a pre-assessment clinic to check you are fit for the operation. At the clinic, you may have blood tests, a blood pressure check and a recording of your heart (ECG).

You will meet a member of the surgical team to talk about the operation. If you are going to have [a stoma](#) after the operation, you will also meet a stoma care nurse, who will explain what is involved.

If you live alone, or care for someone else, you may need help when you go home after surgery. Tell a nurse as soon as possible. They can help to make arrangements for you.

After your operation

After your operation, you will be encouraged to start moving around as soon as possible. Even if you have to stay in bed, it is important to do regular leg movements and deep-breathing exercises. A physiotherapist or nurse will explain these to you.

To reduce the risk of [blood clots](#), you will wear TED stockings. Your nurse may also give you medication to prevent blood clots, as injections under the skin.

After your operation, a nurse will usually help you to get out of bed or to sit up for a short time. Gradually, you will be able to sit or stand up for longer, and to start walking around the ward.

Pain

It is normal to have some [pain](#) or discomfort for a few days after surgery. This can be controlled with painkillers. Immediately after your operation you may have painkillers in the following ways:

- By mouth (orally).
- As an injection.
- Through an epidural. This is a small, thin tube in your back that goes into the space around your spinal cord. It gives you continuous pain relief.
- Through a patient-controlled analgesia pump (PCA pump). The pump is attached to a fine tube ([cannula](#)) in a vein in your arm. You control the pump using a handset that you press when you need more of the painkiller.
- Through a thin tube called a rectus catheter that your surgeon places in your tummy. You may have this kind of catheter if you have the cut downwards from your belly button to your tummy.
- A combination of some of the above.

It is important to tell your doctor or nurse if the pain is not controlled. They can change your painkillers.

Painkillers sometimes slow down your bowel function (cause [constipation](#)). If your bowels are not moving, tell your nurses. They can give you medicines to help.

After an APR, it may be uncomfortable to sit down. This will improve as the wound heals. You may be given a special cushion to help make you feel more comfortable.

Before you can go home, your pain needs to be controlled by tablets. You will get a prescription for painkillers you can take at home as needed.

Wound care

Your wounds may be closed with stitches, clips or staples. A nurse at your GP surgery usually removes these after you go home. Sometimes a district nurse can come to your home to remove them. Some surgeons use dissolving stitches. These do not need to be removed.

Sometimes your surgeon may use a flap of tissue to close the wound, instead of stitches or clips. This is more likely if you have already had radiotherapy. If you have a flap, you will not be able to sit on your bottom for 2 to 4 days after your operation. This is to reduce pressure on your wound and help it heal.

You will only be able to sit for short periods at first. As things improve, you can slowly increase this. You can lie on your side but will still need to change your position regularly. Your doctor or nurse will give you advice about how often to change position when sitting.

You may be given antibiotics to help prevent any wound infection. While you are in hospital and after you go home, tell your nurse or doctor straight away if your wound:

- becomes hot
- becomes painful
- bleeds
- leaks any fluids.

These can be signs that the wound is infected.

Going home

When you can go home will depend on the type of operation you have had. If it was a small operation, you will probably be ready to go home after 2 days.

After a bigger operation, it may take up to 2 weeks.

Before leaving hospital, you will be given an outpatient appointment for your post-surgery check-up. At the appointment, your doctor will talk to you about whether you need any further treatment.

If you have [a stoma](#), the hospital will give you some stoma supplies to take home. But you may also need to order stoma supplies from a pharmacy or specialist supplier. Your stoma care nurse can guide you through this process.

We have more information about [getting support at home after leaving hospital](#).

Getting support

Macmillan is also here to support you. If you would like to talk, you can:

- Call the Macmillan Support Line on [0808 808 00 00](tel:0808 808 00 00).
- [Chat to our specialists online](#).
- Visit our [anal cancer forum](#) to talk with people who have been affected by anal cancer, share your experience, and ask an expert your questions.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our anal cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

S Rao, MG Guren, K Khan et al. Anal cancer: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. 2021. European Society of Medical Oncology. Available from:

www.esmo.org/guidelines/guidelines-by-topic/esmo-clinical-practice-guidelines-gastrointestinal-cancers/anal-cancer

[accessed January 2023].

O Dahl, MP Myklebust, JE Dale et al. Evaluation of the stage classification of anal cancer by the TNM 8th version versus the TNM 7th version. 2020. Acta Oncologica, 59(9), 1016–1023. Available from: www.tandfonline.com/doi/full/10.1080/0284186X.2020.1778180 [accessed December 2022].

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Anal cancer forum

Join our anal cancer forum and chat anonymously to others who understand what you are going through. Our community is available 24/7. It is a safe space where you can share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help